

Special Notice

SUBJECT* AMENDMENT TO STATEMENT OF WORK | REQUEST FOR INFORMATION | RADIATION ONCOLOGY COMMUNITY CARE RURAL/REGIONAL HEALTH PILOT IMPLEMENTATION OF, INTERACTION WITH, AND EVALUATION OF CCRPI-CRSS SYSTEM FOR VA REFERRALS | PILOT PROGRAM FOR ONE

GENERAL INFORMATION

CONTRACTING OFFICE'S ZIP CODE*	21201
SOLICITATION NUMBER*	36C24525Q0433
RESPONSE DATE/TIME/ZONE	04-22-2025 1:00 PM EASTERN TIME, NEW YORK, USA
ARCHIVE	30 DAYS AFTER THE RESPONSE DATE
RECOVERY ACT FUNDS	N
PRODUCT SERVICE CODE*	R410
NAICS CODE*	541990
CONTRACTING OFFICE ADDRESS	Department of Veterans Affairs DEPARTMENT OF VETERANS HEALTH ADMIN VISN 5 SUITE 275 849 INTERNATIONAL DRIVE Baltimore MD 21201
POINT OF CONTACT*	CONTRACTING OFFICER AMY WALTER AMY.WALTER1@VA.GOV

ADDITIONAL INFORMATION

AGENCY'S URL	https://department.va.gov/integrated-service-networks/visn-0
URL DESCRIPTION	VETERANS HEALTH ADMINISTRATION VISN 5
AGENCY CONTACT'S EMAIL ADDRESS	AMY.WALTER1@VA.GOV
EMAIL DESCRIPTION	CONTRACTING OFFICER

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DESCRIPTION

SECTION D - CONTRACT DOCUMENTS, EXHIBITS, OR ATTACHMENTS

D.1 STATEMENT OF WORK

National Radiation Oncology Program (11SPEC22) GRID Community Care Portal Pilot

A. GENERAL INFORMATION

1. Title of Project: **Radiation Oncology Community Care Portal – Rural / Regional Health**
 2. Program Description: The VHA NROP has developed software and database called Health Information and Gateway Exchange (HINGE) and has an ongoing Granular Radiotherapy Information Database (GRID) project dedicated to the prospective ingestion of VA radiotherapy data and its integration with the VA clinical record. However, most Veterans receive radiotherapy in the community (outside of VA facilities). While the VA pays for this care, the treatment and related data that are recaptured by the VA is often severely limited or absent, hampering continuity of care and subsequent care decisions, quality improvement, operations, research initiatives, and the overall integrity of the clinical record. Because these data are generated in the community, the NROP team is seeking to partner with the Contractor as a community care entity to establish a working framework to capture these important data and link them with discrete referral event(s).
 3. Scope of Work: The contract encompasses the supply of detailed, precise radiation oncology data. This data is requisite for the operational goal of ensuring quality of precision oncology care for Veterans. Importantly, most Veterans receive radiotherapy outside of the VA itself, and the details of these treatments are often not available within VA records. The contract will include all information listed in SOW that will pilot both retrospective and prospective data capture for the large number of radiation treatments occurring at community care facilities including VA academic affiliates via the Community Care Radiotherapy Provider Information and Clinical Referral Support System (CCRPI-CRSS).
- The Contractor shall furnish all labor, material, supplies, equipment, qualified personnel, and any other resources necessary to accomplish the deliverables described in the Statement of Work (SOW), except as may otherwise be specified in this contract.
- This project will take place in a single phase over the course of a single year, with the potential for up to 6 months no-cost extension to complete contract deliverables.
4. Performance Period: The period of performance is twelve (12) months-base period with an additional six (6) months no-cost-extension option period from date of award.
 5. Type of Contract: **Firm-Fixed-Price**
 6. Place of Performance: Work will be performed remotely as well as on-site at Contractor's clinic(s).

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B. CONTRACT AWARD MEETING

The Contractor shall not commence performance on the tasks in this SOW until the CO has conducted a kickoff meeting or has advised the Contractor that a kickoff meeting is waived.

C. GENERAL REQUIREMENTS

1. All written deliverables will be phrased in language understood by a general audience without advanced technical knowledge. Statistical and other technical terminology will not be used without providing a glossary of terms.

2. The Contractor shall have 15 calendar days to deliver the final deliverable from date of receipt of the government's comments.

3. The Contractor shall be paid at twelve (12) monthly intervals for work that is consistent with the Tasks and Associated Deliverables described herein. The Contractor shall provide a brief written summary of work to-date, with supporting materials, to facilitate VA review of Contractor's progress. Upon each successful monthly review, VA will disburse interval funds. If review for a given month is deemed unsuccessful, interval funds will be held until subsequent successful review.

4. The Contractor will not be held accountable in interval funding reviews for any failures or delays of the VHA to provide access or permissions to CCRPI-CRSS or any data sources or necessary resources, personnel or IT clearances, etc. provided that the Contractor proactively initiates and expeditiously engages in and fulfills all requests and other requirements as soon as reasonably achievable, and makes full-faith attempts to overcome any barriers encountered, whether through allocation of alternative personnel, seeking of additional support, implementation of alternative technology, modification of communication strategy, etc.

5. This contract will not be construed to modify in any way existing contracts or agreements between Contractor and the VA, and the Contractor will therefore be expected to sustain their existing VA referral handling processes and capabilities (at minimum as a backup/failsafe option in the event of CCRPI-CRSS downtime or issues), in addition to the workflows established as part of this contract.

6. Throughout the work, the Contractor shall collaborate closely with members of the CCRPI-CRSS and broader NROP team and shall be responsive and answerable to NROP leadership or designees regarding matters related to this SOW, and shall maintain and provide to VA a list of active points of contact.

7. The Contractor shall securely transfer all extracted data, reports, and other materials to VA custody via VA-designated system(s) using VA-designated method(s).

8. The Contractor shall participate actively in meetings and discussions as initiated by CCRPI-CRSS team, and will proactively raise questions, concerns, and/or any other discussion items with the CCRPI-CRSS team in a timely fashion. Discussions may include but will not be limited to system assessments, feature engineering, usefulness and usability, barriers, clinical workflow optimization and implementation, rollout strategies, and brainstorming activities. In all cases,

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the Contractor shall furnish qualified personnel with relevance for designated topic areas or specific items or queries.

D. KEY PERSONNEL QUALIFICATIONS

The contractor shall provide qualified personnel who meet the following requirements:

- Practice context: The services covered by this contract shall be rendered at or in support of one or more radiation treatment facilities located in a “rural” setting, defined according to HRSA as non-metropolitan counties, outlying metropolitan counties with no population from an urban area of 50,000 or more people, census tracts with RUCA codes 4-10 in metropolitan counties, census tracts of at least 400 square miles in an area with population density of 35 or fewer people per square mile with RUCA codes 2-3 in metropolitan counties, census tracts with RRS 5 and RUCA codes 2-3 that are at least 20 square miles in area in metropolitan counties (see [How We Define Rural | HRSA](#) for more details); all facilities shall have on-site access to at least one megavoltage medical linear accelerator designed and licensed for therapeutic use in humans; the facility(ies) in aggregate must have commenced operation on or before January 1, 2024 and have delivered treatment to at least 100 Veterans since initiation of operations.
- Radiation Oncologist(s): Physician(s) assigned by the Contractor to perform the services covered by this contract shall have a current license to practice medicine in any State, Territory, or Commonwealth of the United States or the District of Columbia. All licenses held by the key personnel working on this contract shall be full and unrestricted licenses.
 - Qualifications: Board Certification: All contract physician(s) shall be board certified. All continuing education courses required for maintaining certification must be always kept up to date. Documentation verifying current certification shall be provided by the Contractor to the VA COR on an annual basis for each year of contract performance.
- Therapeutic Medical Physicist(s): Therapeutic Medical Physicists (TMP) must be Board Certified and maintain licensure with the Board of Licensure for Professional Medical Physicists, if applicable. Certification must be maintained throughout the contract POP.
 - Qualifications: All contract TMP(s) can either be Master’s or PhD Level.

E. SPECIFIC MANDATORY TASKS AND ASSOCIATED DELIVERABLES

Description of Tasks and Associated Deliverables: In accordance with the Scope of Work outlined in Section A, the Contractor shall:

Task One: Identify and extract data on all Veterans treated at Contractor or Contractor’s affiliates:

1. Creating a comprehensive list of all Veterans referred by the VA for community-based radiotherapy, whose care is paid for by the VA, and who are treated with radiotherapy at one of Contractor’s main and/or affiliate radiation treatment facilities. This list is to include demographic details (name, sex, date of birth, and SSN or partial SSN), billing/referral details (VA referral identifier, billing claims identifier), and consult and

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treatment details (date of initial consultation, attending physician [NPI number], date of simulation, course start and end date(s), delivered fractions, delivered dose, treatment site(s) [anatomical], and location of first treatment [facility]). This is to include an assessment of both current and historical systems for radiotherapy initiated any time prior to the date of initiation of work on the contract.

2. Identifying challenges with and assessing completeness of data extraction
3. Creating a written report detailing methodology and efforts/burden to collect #1, as well as the challenges/barriers/issues identified in #2, including a high-level summary, and any incidental items or findings of note.

Deliverables:

- A. Written report detailing methodology, insights, and dataset (see #3)
- B. Data manifest (see #1)

Task Two: Participate in the development and implementation of the Community Care Radiotherapy Provider Information and Clinical Referral Support System (CCRPI-CRSS):

1. Collaborating with the CCRPI-CRSS team members including other contractors as relevant to establish secure and robust access to the CCRPI-CRSS interface. This activity will also include establishing active user engagement with the system through ID.me authentication, assisting with troubleshooting access process, connection and overall implementation under different IT architectures actively used by the Contractor.
2. Providing, documenting, and organizing iterative feedback to the CCRPI-CRSS team on system utility and usability, design, unmet or only partially met needs, performance and efficiency, issues, and ideas/opportunities for improving efficiency of use and ease of engagement, usefulness of the system, and ensuring data accuracy and completeness.
3. As in #2, providing, documenting, and organizing iterative feedback to the CCRPI-CRSS team on usability of case report forms, their design, performance and efficiency and technical issues, and ideas/opportunities for improving efficiency of use and ease of engagement, and ensuring data accuracy and completeness.
4. Developing, evaluating/studying, and iterating upon clinical workflow implementation of referral intake, distribution/processing/completion, and ultimately data return process via CCRPI-CRSS, potentially with engagement of multiple stakeholders/roles, to arrive at optimal implementation of CCRPI-CRSS and case report form completion in workflow. This process should be well-documented and should capture the time/effort demands at each step of the process, with formative feedback and reporting provided to CCRPI-CRSS team throughout.
5. Monitoring and tracking incoming VA radiotherapy referrals to manually capture the prospective treatment of at least **25** individual Veterans (if **25** Veterans are processed prior to contracted end date, the Contractor will be expected to continue engagement with CCRPI-CRSS for additional Veterans through the end of the contract). This will also include timely completion of a patient-specific case report form following initial

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consultation (ideally within 10 days after completed consultation) and additional patient-specific case report forms following completion of treatment, where relevant/delivered (ideally within 10 days after completion of treatment). All planning and treatment-related DICOM-RT data associated with measurable radiotherapy dose will be uploaded to CCRPI-CRSS; these include but may not be limited to: initial simulation images (if any) and registration details (if any) used for treatment planning purposes (plus any additional re-simulation images along with their image registration details), any/all RTPLAN data receiving measurable radiotherapy dose (including plan sum for aggregate delivered dose, where relevant and feasible), any/all RTDOSE grid(s) associated with RTPLAN at native/as-calculated/as-planned resolution, any/all RTSTRUCT structure sets associated with a RTPLAN receiving treatment, any/all RTRECORD objects verifying measurable radiotherapy dose (i.e. record and verify data confirming dates and doses and other details of treatments), any/all RTIMAGE objects associated with treatment verifications/setups; all DICOM-RT data should retain all original identifiers (i.e. no record anonymization) and data, and should include calculated dose volume histogram (DVH) data for all structures. Structure nomenclature will adhere to TG-263 standards where relevant. Completed CCRPI-CRSS patient-specific case report forms will also be uploaded to CCRPI-CRSS. Other clinical data, including a treatment summary note per standard workflow, may also be uploaded to CCRPI-CRSS.

6. Creating a written report thoroughly detailing all aspects of #1-5, including a high-level summary, list/manifest of Veterans treated, and any incidental items or findings of note. This report shall include a final assessment of system usefulness and usability, and optimal clinical workflow and implementation, including accounting of time/efforts and details of any barriers or challenges for implementation and/or broader rollout.

Deliverables:

- A. Uploaded prescription and treatment datasets (see #5)
- B. Written report (see #6) detailing methodology, implementation workflow including pros/cons of changes over time, all project feedback, insights, and dataset

Task Three: Evaluate the feasibility of retrospective data extraction using CCRPI-CRSS:

1. **Selecting a set of 100 patient radiotherapy courses randomly from among the list of historical VA patient treatments identified in Task One.** For each of these cases, a set of case report forms will be completed (as in Task Two #5), and all DICOM-RT data associated with treatment will be extracted (also as in Task Two #5) where possible, with all data being uploaded to CCRPI-CRSS in coordination with the CCRPI-CRSS team.
2. Measuring and otherwise evaluating the time and efforts required to curate, extract, and ultimately upload the data as per #1, as well as assessing any barriers encountered preventing or otherwise hampering aspects of archival data retrieval. Assessments should include estimation of data element specific levels of effort and processes to complete specific aspects of case report form(s).

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3. Creating a written report detailing methodology and efforts/burden to collect #1, as well as the challenges/barriers/issues identified in #2, including a high-level summary, and any incidental items or findings of note.

Deliverables:

- A. Uploaded prescription and treatment datasets (see #1)
- B. Written report (see #3) detailing methodology (including list of selected patients), implementation workflow, quality/completeness assessment of retrospective data extraction, and any/all project feedback, insights, and data.

F. CHANGES TO STATEMENT OF WORK

Any changes to this SOW shall be authorized and approved only through written correspondence from the CO. A copy of each change will be kept in a project folder along with all other products of the project. Costs incurred by the Contractor through the actions of parties other than the CO shall be borne by the Contractor.

G. REPORTING REQUIREMENTS

At least every month meet directly with CCRPI-CRSS team to report and discuss current progress and pitfalls (meeting can be virtual, in person, via phone, or any combination thereof).

Every month provide written status reports detailing the status and progress of data identification/retrieval, portal functionality, clinical workflow implementation, a summary of current challenges and needs, and anticipated next steps and goals over subsequent month interval.

Within 1 months of completion of this contract, the final extracted dataset for historically referred Veterans along with all keys, accompanying metadata, and descriptive README documentation shall be securely transferred to VA NROP custody.

Within 1 month of completion of this contract a final document describing the process and methods for retrospective data retrieval (including case report form completion and DICOM-RT extraction), study team individual roles/contributions, findings and insights including needs assessment and both pitfalls and actual or potential solutions, and an accounting/analysis of data completeness and integrity shall be provided.

Within 1 month of completion of this contract, the final extracted datasets of radiation treatments for all **twenty-five (25) prospective** Veteran referrals will have been securely transferred via CCRPI-CRSS.

Within 1 month of completion of this contract, the final extracted datasets of radiation treatments for all **one hundred (100) retrospectively** assessed Veteran referrals will have been securely transferred via CCRPI-CRSS.

Within 1 month of completion of this contract a final document describing the process and methods for CCRPI-CRSS access and implementation, study team individual roles/contributions, findings and insights including needs assessment and both pitfalls and

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actual or potential solutions, detailed feedback history, and an assessment of usability and usefulness shall be provided.

All written reports should be submitted electronically to the COR. The COR will provide these reports to the NROP CCRPI-CRSS team for review, before certifying invoices.

H. SECURITY - The C&A requirements do not apply, and a Security Accreditation Package is not required.

Information System Security

The Contractor will be provided access to secure web-based VA data exchange portal for all required development, implementation, and testing support. The Contractor will be expected to maintain its own information system security as authorized for patient care.

The COTR will:

- Ensure that a Contractor Security Control Assessment (CSCA) is completed within 30 days of contract approval and again a year later, as relevant.
- Ensure that the CSCA is sent to the ISO and the OCS Certification Program Office for review to ensure that appropriate security controls are being implemented in service contracts.
- Ensure a copy of the CSCA is maintained in the Security Management and Reporting Tool (SMART) database. COTR will provide a copy of the completed CSCA to ISO for uploading into SMART database.

CONTRACTOR Personnel Security

Position Sensitivity – The position sensitivity has been designated as

Low Risk – Tier 1 category

Background Investigation – Background investigations will not be needed.

Historical contract: Cone Health 36C24524P0734; UCSD 36C24524P08934; OHSU 36C26023P0401

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REQUEST FOR INFORMATION

THIS IS A REQUEST FOR INFORMATION ANNOUNCEMENT AND IT IS FOR INFORMATION & PLANNING PURPOSES ONLY; IT IS NEITHER A SOLICITATION ANNOUNCEMENT, NOR A REQUEST FOR PROPOSALS OR QUOTES AND DOES NOT OBLIGATE THE GOVERNMENT TO AWARD A CONTRACT. IF A VENDOR RESPONDS TO THIS INTENT TO SOLE SOURCE, THE VENDOR MAY NOT OR MAY RECEIVE A RESPONSE. THE PURPOSE OF THIS REQUEST FOR INFORMATION ANNOUNCEMENT IS FOR MARKET RESEARCH, IN ORDER TO MAKE APPROPRIATE ACQUISITION DECISIONS, AND TO GAIN KNOWLEDGE OF POTENTIALLY QUALIFIED SERVICE-DISABLED VETERAN OWNED SMALL BUSINESSES, VETERAN OWNED SMALL BUSINESSES, 8(A), HUBZONE AND OTHER SMALL BUSINESSES WHO STATE THEY ARE CAPABLE OF PERFORMING THE WORK. ****A contract award will NOT result from this Notice.****

IN THE EVENT THAT A VENDOR WISHES TO RESPOND TO THIS REQUEST FOR INFORMATION, THEN DOCUMENTATION OF TECHNICAL EXPERTISE MUST BE PRESENTED IN SUFFICIENT DETAIL FOR THE GOVERNMENT TO DETERMINE THAT THE COMPANY POSSESSES THE NECESSARY FUNCTIONAL AREA EXPERTISE AND EXPERIENCE TO COMPETE FOR THIS ACQUISITION.

****VENDOR RESPONSE SHALL INCLUDE THE BELOW****

Responses to this REQUEST FOR INFORMATION shall include company name, address, point of contact, phone number, point of contact e-mail, DUNS Number, Cage Code, size of business pursuant to North American Industrial Classification System ([NAICS 541990](#)), (size standard of \$19.5M, per SBA Table of Small Business Size Standards January 1, 2022), ([PSC R410](#)) AND whether your company has special socio-economic status (i.e., SDVOSB, VOSB, Small Business, HUBZone, etc.).

(A) COMPANY NAME

(B) ADDRESS

(C) POINT OF CONTACT

(D) PHONE, FAX, AND EMAIL

(E) DUNS NUMBER

(F) CAGE CODE

(G) TAX ID NUMBER

(H) TYPE OF SMALL BUSINESS, E.G. SERVICES DISABLED VETERAN OWNED SMALL BUSINESS, VETERAN-OWNED SMALL BUSINESS, 8(A), HUBZONE, WOMEN OWNED SMALL BUSINESS, SMALL DISADVANTAGED BUSINESS, OR SMALL BUSINESS HUBZONE

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BUSINESS, (I) WILL YOUR COMPANY BE PERFORMING THE WORK? OR DO YOU PLAN ON SUB-CONTRACTING OUT THE WORK?

(I) Is your company considered small under the NAICS code identified under this RFI? (Size standard of = or < \$30M, per SBA Table of Small Business Size Standards August 19, 2019)

(J) Does your company have an FSS contract with GSA or the VA NAC or a GWAC contract holder with any other federal agency? If so, please provide the contract number.

(K) If your company holds a FSS GSA/VA NAC contract other federal GWAC contract holder, are the services requested available on your schedule/contract?

(L) General pricing of your products is encouraged. Pricing will be used for the purpose of market research only. It will not be used to evaluate for any type of award.

AND

(J) ***WILL YOUR COMPANY BE DOING ALL OF THE WORK? IF NOT, WILL YOUR COMPANY BE SUB-CONTRACTING OUT THE WORK?***MUST ANSWER*** MUST PROVIDE A CAPABILITY STATEMENT THAT ADDRESSES THE ORGANIZATIONS QUALIFICATIONS AND ABILITY TO PERFORM AS A CONTRACTOR FOR THE WORK DESCRIBED BELOW. IF THE VENDOR IS AN SDVOSB/VOSB, BE PREPARED TO DEMONSTRATE HOW IT WILL NOT SUB-CONTRACT THE WORK BY MORE THAN 51%, PER VAAR 852.219-75, VAAR 819.7011(b) Contract clauses. (DEVIATION)

*****IF THERE ARE ANY QUESTIONS ABOUT THIS REQUEST FOR INFORMATION, CONTACT THE CONTRACTING OFFICER AT**
AMY.WALTER1@VA.GOV.***